Fax cover sheet

Marigold Canyon Hospital (fictional)

Case Management Department

FAX TRANSMITTAL

To: Illumination Health + Home - Recuperative Care Intake
Fax: 555-0199
From: Denise Park, RN CM
Phone: 555-0192
Date: 09/09/2026 13:30
Pages (including cover): 12
Re: Recuperative care referral
Urgent [X] For Review [X] Please Reply [X]

Message

61M admitted with colitis, improving on oral medication.
Independent, AOx4. Requesting d/c 09/10/2026.

CONFIDENTIALITY NOTICE

This facsimile contains confidential health information. If received in error, notify sender and destroy.

-- FICTIONAL DEMO RECORD - not a real patient --

IHH Recuperative Care Referral Form

ILLUMINATION HEALTH + HOME

Recuperative Care Referral Form (RC0725-SH)

Referring Party

Hospital: Marigold Canyon Hospital (fictional)
Department/Floor: 5 West Medical
Social Worker/Case Manager/Nurse: Denise Park, RN CM
Phone: 555-0192
Email: dpark@marigoldcanyon.example
Nursing Station: 555-0193
Authorized By: Dr. V. Shah

Patient Information

Name: Walter Doe
MRN: MC-2265098
Birth Date: 11/27/1964
Gender: M
Ethnicity: White
SSN (last 4): 0000
Insurance: Orange Coast Health Plan (fictional) Medi-Cal
Preferred Language: English
Housing Situation:
Currently Unhoused [X]
4+ episodes of homelessness in the past 3 years [ ]

Hospital Admission Details

Chief Complaint/Admitting Diagnosis: Colitis / diarrhea - improving
Admit Date: 09/05/2026
Any Wounds: Yes [ ] No [X]
Number/Location/Size/Stage:
Independent with Wound Care: Yes [ ] No [ ]
Home Health Ordered: Yes [ ] No [X]
PPD/TB Test or Chest X-Ray: Date: 09/05/2026 Result: CXR negative

History of Health & Living Situation

O2: Yes [ ] No [X]
Self Administer All Meds: Yes [X] No [ ]
Continent of Bowel & Bladder: Yes [X] No [ ]
Colostomy/Ileostomy: Yes [ ] No [X]
Foley Catheter: Yes [ ] No [X]
Diabetes: Yes [ ] No [X]
Insulin: Yes [ ] No [X]
Diabetes Oral Meds: Yes [ ] No [X]
Active Infection: Yes [ ] No [X]
Ambulatory: Yes [X] No [ ]
Assistive Device: Yes [ ] No [X]
Hospice: Yes [ ] No [X]
Car: Yes [ ] No [X]
Service Animal: Yes [ ] No [X]

Behavioral and Mental Health Status

Any limitations or challenges: No [X] Yes [ ]

Invented for the Claude Impact Lab demo. Names, facilities and numbers are fictional.

CalAIM Community Supports authorization request

CalAIM COMMUNITY SUPPORTS - AUTHORIZATION REQUEST

Service: Recuperative Care (Medical Respite)
Health Plan: Orange Coast Health Plan (fictional) - Medi-Cal

Member

Member Name: Walter Doe
DOB: 11/27/1964
Member ID: OCH-71502388

Referring Provider

Facility: Marigold Canyon Hospital (fictional)

Clinical

Primary Diagnosis: Enterocolitis due to Clostridioides difficile
ICD-10: A04.72
Member is experiencing homelessness [X]
Member at risk of hospitalization/re-hospitalization [X]
Requested Start Date: 09/10/2026
Requested Days: 14

Plan Use Only

Status: Pending review

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Patient face sheet

Marigold Canyon Hospital (fictional) - PATIENT FACE SHEET

Patient: Walter Doe
MRN: MC-2265098
Account #: 99265098
DOB: 11/27/1964
Sex: Male
Preferred Language: English
Ethnicity: Not Hispanic or Latino
Marital Status: Single
Address: NO FIXED ADDRESS (homeless)
Mailing: 1234 Example Ave, Santa Ana, CA 92700 (shelter mail drop)
Phone: 555-0100 (message)

Encounter

Admit Date/Time: 09/05/2026 11:45
Admit Type: Inpatient
Unit/Bed: 5W-507
Attending: Vikram Shah, MD

Insurance

Primary: Orange Coast Health Plan (fictional) - Medi-Cal
Subscriber: Self

Emergency Contact

None listed

Isolation

Isolation status: CONTACT ENTERIC PRECAUTIONS (C. difficile) - ordered 09/05/2026

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History and Physical - page 1

HISTORY AND PHYSICAL

Date: 09/05/2026 13:00

CC

Watery diarrhea x 5 days, abdominal cramping

HPI

61 y/o M, unhoused (sleeping at a bus shelter, uses day center showers), completed 10-day course of clindamycin for a dental abscess ending ~1 week ago. Now with 8-10 watery stools per day, lower abdominal cramping, low-grade fever, poor PO intake, lightheaded on standing. No blood in stool.

ED: T 100.6F, HR 108, BP 102/64, WBC 16.8, Cr 1.4 (baseline unknown).

PMH

- HTN (not on meds)
- Dental abscess 08/2026

Meds PTA

Clindamycin (completed)

Allergies

NKDA

Social

Unhoused x 3 years. Former smoker. Denies EtOH or drugs.

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History and Physical - page 2

Exam

Gen: tired, dry mucous membranes, AOx4
Abd: soft, mild diffuse lower abdominal tenderness, no rebound, no distension, active bowel sounds

Data

C. difficile PCR: POSITIVE. C. difficile toxin EIA: POSITIVE.
CT A/P: mild colonic wall thickening consistent with colitis, no megacolon, no perforation

Assessment/Plan

1. C. difficile infection, non-severe to moderate (WBC 16.8, Cr 1.4) after recent clindamycin
- Oral vancomycin 125 mg PO four times daily x 10 days
- CONTACT ENTERIC PRECAUTIONS - soap and water hand hygiene, dedicated bathroom
2. AKI due to dehydration - IV fluids
3. Dispo: unhoused, CM consult
Signed: Vikram Shah, MD 09/05/2026 14:20

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Hospitalist progress note

HOSPITALIST PROGRESS NOTE - HD#5

Date: 09/09/2026 08:30
S: Cramping improved. Still having loose stools, 5 in the last 24 hours, watery to mushy. Eating 75% of meals.
O: T 98.8F HR 88 BP 118/74 RR 16 SpO2 98% RA. AOx4. Abd soft, mild LLQ tenderness.
Labs: WBC 10.9, Cr 0.9.
A/P: C. difficile infection, improving but ongoing diarrhea.
- Continue oral vancomycin 125 mg QID, day 5 of 10 (through 09/14/2026)
- Remains on contact enteric precautions. Per infection prevention, precautions continue until no diarrhea for at least 48 hours.
- AKI resolved
Dispo: CM exploring recuperative care; pt otherwise medically improving.
Signed: Vikram Shah, MD

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Nursing note and stool log

NURSING NOTE

Date: 09/09/2026 06:30
Isolation: Contact Enteric Precautions sign on door. Pt using dedicated bathroom.
Pt AOx4, calm and cooperative. Denies SI.
Ambulating independently in room. Toileting independently, one episode of urgency, made it to bathroom.
Self-administers oral vancomycin with RN observation.

STOOL LOG (last 48h)

Date/Time | Consistency (Bristol) | Initials
09/07 22:10 | 7 watery | KL
09/08 04:30 | 6 loose | KL
09/08 09:15 | 7 watery | TR
09/08 13:40 | 6 loose | TR
09/08 19:20 | 6 loose | TR
09/09 01:05 | 6 loose | KL
09/09 05:50 | 7 watery | KL
Signed: K. Lim, RN

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Infection prevention note

INFECTION PREVENTION NOTE

Date: 09/09/2026 11:00
Organism: Clostridioides difficile (PCR + / toxin +), collected 09/05/2026
Isolation status: ACTIVE - Contact Enteric Precautions
Criteria to discontinue: no diarrhea (fewer than 3 unformed stools in 24h) for at least 48 hours. Criteria NOT met as of 09/09/2026: 5 unformed stools in last 24h.
Recommend continued precautions for remainder of stay and notification of any receiving facility.
Signed: M. Ortega, RN, CIC (Infection Prevention)

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Physical therapy evaluation

PHYSICAL THERAPY EVALUATION

Date: 09/07/2026 (in isolation room / hallway with precautions)
Bed mobility: Independent
Transfers: Independent
Gait: 300 ft without assistive device, independent, steady
Stairs: not tested

Recommendation

No skilled PT needs. No DME.
Signed: E. Haas, PT

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Vitals, labs and microbiology

VITAL SIGNS - LAST 5 SETS

Date/Time | Temp (F) | HR | BP | RR | SpO2 | O2
09/07 20:00 | 99.0 | 92 | 116/72 | 16 | 98% | RA
09/08 08:00 | 98.7 | 90 | 114/70 | 16 | 98% | RA
09/08 16:00 | 98.9 | 88 | 120/76 | 16 | 97% | RA
09/09 00:00 | 98.6 | 86 | 118/72 | 16 | 98% | RA
09/09 08:30 | 98.8 | 88 | 118/74 | 16 | 98% | RA

LABS

09/05 | WBC 16.8 H | Hgb 13.2 | Plt 301 | Na 134 L | K 3.3 L | Cr 1.4 H | Lactate 1.6
09/09 | WBC 10.9 H | Hgb 12.7 | Na 138 | K 3.9 | Cr 0.9

MICRO

09/05 C. difficile PCR: POSITIVE
09/05 C. difficile toxin EIA: POSITIVE
09/05 MRSA nares PCR: Negative
09/09 07:00 SARS-CoV-2 rapid antigen: Negative

IMAGING

09/05 CXR: no acute process
09/05 CT A/P: colitis, no megacolon

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Discharge medication list (draft)

DISCHARGE MEDICATION LIST - DRAFT

Patient: Walter Doe MRN: MC-2265098

- Vancomycin 125 mg PO four times daily through 09/14/2026
- Acetaminophen 650 mg PO every 6 hours PRN pain or fever

Avoid

- Loperamide and other anti-diarrheals (C. difficile)

Stopped

- IV fluids (09/08)

Follow up

- PCP: Example Community Clinic (fictional) 555-0194, within 1 week of discharge

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